

# **ClaimGuard Business Plan**

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### **Executive summary**

ClaimGuard is a denial-prevention and appeal-operations SaaS for independent medical practices. The product scans high-value claims and prior authorization requests before submission, predicts payer-specific denial risk, assigns the missing documentation as work items, and produces appeal packets when a denial happens.

The wedge is intentionally narrow: specialty practices with expensive, documentation-heavy services such as cardiology imaging, orthopedic imaging, infusion therapy, home health, and sleep medicine. These practices feel the pain of payer automation but rarely have enterprise revenue-cycle teams or custom EHR integrations.

ClaimGuard starts as a lightweight workflow layer that works from structured forms and CSV exports, then expands into EHR/clearinghouse integrations and CMS prior-authorization API readiness.

### **Market problem**

Healthcare providers are losing margin to administrative claim review, denial rework, and prior authorization delays.

- Premier reported that claims adjudication cost providers \$25.7B in 2023, a 23% increase from the prior year, and said 70% of denials were ultimately overturned only after costly review cycles.
- CMS finalized interoperability and prior authorization rules that require impacted payers to improve API-based access, report usage metrics, and support faster, more transparent prior authorization workflows.

- HFMA has emphasized that denial prevention depends on standardized categories, trend data, and root-cause workflows, not only post-denial firefighting.
- The American Hospital Association cited nearly 15% initial denial rates for private-payer claims and \$19.7B spent by hospitals and health systems in 2022 trying to overturn denials.

Sources: - Premier: <https://premierinc.com/newsroom/policy/claims-adjudication-costs-providers-257-billion-18-billion-is-potentially-unnecessary-expense> - CMS final rule: <https://www.cms.gov/initiatives/burden-reduction/overview/interoperability/policies-regulations/cms-interoperability-prior-authorization-final-rule-cms-0057-f> - HFMA denial metrics: <https://www.hfma.org/guidance/standardizing-denial-metrics-revenue-cycle-benchmarking-process-improvement/> - AHA market scan: <https://www.aha.org/aha-center-health-innovation-market-scan/2024-04-02-payer-denial-tactics-how-confront-20-billion-problem>

## Customer and ICP

Primary ICP: - Independent specialty practice groups with 5-75 providers. - Monthly claim volume of 1,000-20,000 claims. - High-value procedures where denials are financially material. - Thin billing teams using EHR exports, clearinghouse portals, spreadsheets, and manual payer calls.

Economic buyer: - Practice administrator, RCM manager, CFO, or physician-owner.

Daily users: - Billing specialists, prior authorization coordinators, referral coordinators, and practice managers.

Beachhead specialties: - Cardiology and cardiac imaging. - Orthopedics and imaging-heavy musculoskeletal care. - Infusion and biologics. - Home health recertification. - Sleep medicine.

## Solution

ClaimGuard has four product loops.

1. Pre-submission risk scan
  - User enters or imports payer, CPT, service, amount, deadline, and documentation notes.
  - ClaimGuard scores denial risk and identifies payer-specific missing evidence.
  - Risk is explainable: payer category, high-dollar exposure, missing evidence, deadline pressure.
2. Documentation work queue
  - Missing documentation becomes assigned work items.

- Practice managers can see at-risk cases, owners, deadlines, expected revenue exposure, and status.
  - The app prioritizes claims with the highest risk-adjusted dollars.
3. Appeal packet builder
- For denied claims, ClaimGuard generates the packet checklist: clinical notes, payer rule, coding rationale, timeline, physician attestation, and submission proof.
  - It tracks whether the packet is appeal-ready, submitted, or recovered.
4. Business intelligence
- Admin dashboard shows denial-risk distribution, recovered revenue, bottlenecks, payer trends, and team throughput.
  - This becomes the renewal story: recovered revenue, fewer days in appeal, and cleaner first-pass claims.

## **Product shipped in this arena build**

The deployed demo includes: - Landing page that explains the business and market wedge. - Demo authentication with practice and admin accounts. - Practice dashboard with live metrics, work queue, payer playbook, claim scanner, risk scoring, and status advancement. - Admin dashboard with funnel metrics, business signals, recovered revenue, and the same live work queue. - Local browser persistence using `localStorage`, so new cases and status changes survive refreshes in the demo environment.

## **Competitive landscape**

Competitor categories:

- Enterprise RCM platforms: Optum, Waystar, Experian Health, R1, FinThrive.
  - Strength: broad integrations and deep enterprise contracts.
  - Weakness: long implementation cycles and pricing/complexity mismatched to independent practices.
- Clearinghouses and EHR modules: Availity, Change Healthcare/Optum, athenahealth, eClinicalWorks modules.
  - Strength: already in workflow.
  - Weakness: often focus on eligibility/submission rather than payer-specific denial prevention and appeal orchestration.
- Services-led billing companies.
  - Strength: done-for-you operations.
  - Weakness: labor-intensive, opaque, and not a software-margin product.

ClaimGuard positioning: - Lighter than enterprise RCM. - More opinionated than spreadsheets. - More prevention-oriented than denial cleanup services. - Designed for fast implementation by practices that can export claims today.

## **Business model**

Pricing: - Starter: \$499/month per practice, includes 3 users and 1,000 scanned claims/month. - Growth: \$1,499/month, includes 15 users, 10,000 scanned claims/month, payer playbooks, CSV imports, and admin analytics. - Performance add-on: 0.9% of recovered revenue above baseline, capped by contract. - Implementation: \$1,500 one-time onboarding for mapping payer/CPT workflows and historical denial categories.

Why this pricing works: - If one high-value appeal is recovered each month, the software can pay for itself. - The base subscription keeps revenue predictable. - The performance fee aligns incentives without making the practice hand over full billing operations.

## **Go-to-market**

Phase 1: founder-led pilots - Target 20 specialty practices in Texas, Arizona, Ohio, Florida, and California. - Offer a 30-day denial-risk audit using exported claim and denial data. - Convert audit into a paid pilot with ROI report.

Phase 2: channel partnerships - Partner with boutique billing companies serving independent specialty practices. - Give billing partners a white-label dashboard and revenue share. - Use specialty societies, RCM consultants, and practice administrator communities as distribution.

Phase 3: integration-led expansion - Add EHR/clearinghouse import connectors. - Add payer API readiness modules as CMS interoperability requirements mature. - Move from prevention workflow into automated packet generation and payer-specific analytics.

## **Financial projections**

Assumptions: - Average subscription revenue per practice: \$1,100/month in year 1, \$1,650/month in year 2, \$2,050/month in year 3. - Implementation fee averages \$1,000 net after discounts. - Gross margin: 78% year 1, 83% year 2, 86% year 3. - Logo churn: 16% year 1, 11% year 2, 8% year 3.

Year 1: - 35 paying practices. - Subscription ARR exiting year: \$462K. - Implementation revenue: \$35K. - Total recognized revenue estimate: \$265K.

Year 2: - 155 paying practices. - Subscription ARR exiting year: \$3.07M. - Total recognized revenue estimate: \$1.78M.

Year 3: - 520 paying practices. - Subscription ARR exiting year: \$12.79M. - Total recognized revenue estimate: \$7.4M.

Long-term upside: - 5,000 practices at \$2,000/month is \$120M ARR before performance revenue. - The category can expand into payer scorecards, outsourced appeal networks, and embedded financing for delayed reimbursements.

## **Roadmap**

0-30 days: - Harden auth and multi-tenant database. - Add CSV import/export and audit trail. - Add denial category taxonomy based on HFMA-style definitions.

31-90 days: - Pilot with 5-10 specialty practices. - Add appeal packet PDF generation. - Add payer-specific templates for top five payer/service combinations. - Build ROI report and monthly executive summary.

3-6 months: - Integrate with clearinghouse exports. - Add team assignments, SLA notifications, and document checklist completion. - Add analytics by payer, CPT, physician, and denial category.

6-12 months: - EHR integrations for top beachhead systems. - Prior authorization API readiness module. - AI-assisted clinical letter drafting with human approval and compliance controls.

## **Risks and mitigations**

Risk: EHR integration friction. Mitigation: start with CSV/export workflows and high-value manual queues.

Risk: Complex healthcare compliance. Mitigation: begin as workflow software without storing unnecessary PHI in the MVP; add BAA, audit logs, and access controls before production pilots.

Risk: Enterprise incumbents copy features. Mitigation: win on specialty-specific templates, speed, and independent-practice affordability.

Risk: Denial rules change by payer. Mitigation: maintain a payer playbook library updated from customer outcomes and public policy updates.

## **Why now**

Payers are automating review and prior authorization, providers are reporting higher administrative cost, and CMS rules are pushing the ecosystem toward more transparent, API-enabled prior authorization. Independent practices need a practical bridge now: a product that works with exports and staff workflows today, then becomes the integration layer tomorrow.

## **Demo credentials**

- Practice dashboard: practice@claimguard.demo / DemoPass!2026
- Admin dashboard: admin@claimguard.demo / AdminPass!2026